

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase II Clinical Trial to Study the Efficacy and Safety of Pembrolizumab (MK-3475) and Pembrolizumab in Combination With Other Investigational Agents in Subjects With High Risk Non-muscle-Invasive Bladder Cancer (NMIBC) Unresponsive to Bacillus Calmette-Guerin (BCG) Therapy
Short Title:	Study of Pembrolizumab (MK-3475) and Pembrolizumab With Other Investigational Agents in Participants With High Risk Non-muscle Invasive Bladder Cancer (MK-3475-057/KEYNOTE-057)
Protocol Number:	79050007538655331
NCT Number:	NCT02625961
Sponsor Name:	Merck
Investigational Product:	pembrolizumab
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Cohort A: Complete Response (CR) Rate of High-Risk Non-Muscle Invasive Bladder Cancer (NMIBC): The CR rate of high-risk NMIBC is defined as the proportion of participants in the analysis population whose tumors are in response to treatment and who are then free of high-risk NMIBC or worse per central pathology and radiology review. 2. Cohort B: 12-month Disease-Free Survival (DFS) Rate of High-Risk NMIBC: DFS of high-risk NMIBC is defined as the time from the first dose to the first occurrence of high-risk NMIBC or worse per central pathology and radiology review. The 12-month DFS rate is the percentage of participants remaining free from high-risk NMIBC or worse at 12 months. 3. Cohort A and B: Number of Participants Who Experience an Adverse Event (AE): An AE is defined as any untoward medical occurrence in a participant or clinical investigation subject administered a pharmaceutical product and which does not necessarily have to have a causal relationship with this treatment.
Secondary Obj.:	1. Cohort A: CR Rate of Any Disease 2. Cohort A: CR Rate of High-Risk NMIBC in Programmed Cell Death 1 (PDL-1) Positive Participants 3. Cohort A: Duration of Response (DOR)

2.2 Background & Rationale

In this study, participants with high risk non-muscle-invasive bladder cancer (NMIBC) unresponsive to Bacillus Calmette Guerin (BCG) therapy and who are considered ineligible for or have refused to undergo radical cystectomy, will receive pembrolizumab therapy or pembrolizumab in combination with other investigational agents. The primary study hypothesis is that treatment with pembrolizumab will result in a clinically meaningful response.

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3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE2, Status: ACTIVE_NOT_RECRUITING

Target N: 320

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Histologically-confirmed diagnosis of high risk non-muscle-invasive (T1, high grade Ta and / or carcinoma in situ [CIS]) transitional cell carcinoma of the bladder (mixed histology tumors allowed if transitional cell histology is predominant histology).
- * Fully resected disease at study entry (residual CIS acceptable)
- * BCG-unresponsive high risk non-muscle-invasive bladder cancer after treatment with adequate BCG therapy
- * Ineligible for radical cystectomy or refusal of radical cystectomy
- * Available tissue from a newly obtained core biopsy of a tumor lesion not previously irradiated
- * Eastern Cooperative Oncology Group (ECOG) performance status of 0, 1, or 2
- * Adequate organ function
- * Female participants of childbearing potential have a negative urine or serum pregnancy test and must be willing to use an adequate method of contraception
- * Male participants must be willing to use an adequate method of contraception

Exclusion criteria:

- * Centrally assessed muscle-invasive, locally advanced nonresectable, or metastatic urothelial carcinoma (i.e., T2, T3, T4, and / or stage IV)
- * Centrally assessed concurrent extra-vesical (i.e., urethra, ureter, or renal pelvis) non-muscle invasive transitional cell carcinoma of the urothelium
- * Currently participating or has participated in a study of an investigational agent and received study therapy or received investigational device within 4 weeks prior to the first dose of study treatment
- * Received intervening intravesical chemotherapy or immunotherapy from the time of most recent cystoscopy / Transurethral Resection of Bladder Tumor (TURBT) to starting study treatment
- * Received prior chemotherapy, targeted small molecule therapy, or radiation therapy within 2 weeks prior to starting study treatment or not recovered from adverse events due to a previously administered agent
- * Known additional malignancy that is progressing or requires active treatment excepting basal cell carcinoma of the skin, squamous cell carcinoma of the skin that has undergone potentially curative therapy or in situ cervical cancer. A history of prostate cancer that was treated with definitive intent (surgically or through radiation therapy) is acceptable provided that the following criteria are met: Stage T2N0M0 or lower; Gleason score ≤ 7 and prostatic-specific antigen (PSA) undetectable for at least 1 year while off androgen deprivation therapy that was either treated with definitive intent or untreated in active surveillance that has been stable for the past year prior to study allocation
- * Active autoimmune disease that has required systemic treatment in the past 2 years
- * Evidence of interstitial lung disease or active non-infectious pneumonitis

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- * Active infection requiring systemic therapy
- * Pregnant or breastfeeding, or expecting to conceive within the projected duration of the trial through 120 days after the last dose of study treatment
- * Prior therapy with an anti-programmed cell death 1 (PD-1), anti-PD-ligand 2 (L2) agent, or with an agent directed to another co-inhibitory T-cell receptor
- * Known human immunodeficiency virus (HIV)
- * Known active Hepatitis B or C infection
- * Received a live virus vaccine within 30 days of planned start of study treatment
- * Has had an allogeneic tissue/solid organ transplant

4.2 Exclusion Criteria

Not Provided

11. Study Identifiers & Governance

Primary ID: 79050007538655331

Registry Number: NCT02625961

Sponsor: Merck

Study Title: Study of Pembrolizumab (MK-3475) and Pembrolizumab With Other Investigational Agents in Participants With High Risk Non-muscle Invasive Bladder Cancer (MK-3475-057/KEYNOTE-057)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____